Diabetes Insipidus *Prodigy Anesthesia*

Diabetes insipidus results from the inadequate secretion of antidiuretic hormone (ADH) or the inability of the nephrons to respond adequately to ADH (nephrogenic diabetes insipidus). Inadequate secretion of ADH results in polydipsia, hypernatremia, and a high output of poorly concentrated urine. The most serious immediate effects of this disorder are hypovolemia and hypernatremia.

Typical causes include damage to the pituitary gland from intracranial trauma, pituitary lesions, or surgery. Treatment intraoperatively comprises of administration of aqueous ADH between 100-200 milliunits/hour and administration of an isotonic crystalloid. Other treatments regimens that may be seen in the perioperative patient include oral chlorpropamide (a hypoglycemic agent), intranasally administered DDAVP, or intramuscularly administered vasopressin tannate. Interestingly, clofibrate, an oral hypolipidemic agent is also capable of increasing ADH secretion.

Nagelhout JJ, Plaus K. Nurse Anesthesia. 5th ed. St. Louis, MO: Elsevier Saunders Company; 2014: 846.

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